

CLINICAL REASONING CASES

Relational & extended-abstract prompts

Every other document in this system is unistructural or multistructural, in SOLO-taxonomy terms -- recall a fact, identify a point, match a pathway. That's necessary but not sufficient: real clinical reasoning is RELATIONAL (connecting multiple facts into one coherent picture) and EXTENDED ABSTRACT (generalizing the pattern to a case you haven't seen before). These five cases are built for that. There's no multiple choice here -- write or think through your own answer FIRST, then compare it to the model answer. The gap between your reasoning and the model's is more informative than whether you got the "right" point.

CASE 1

SOLO level: Relational

A patient presents with low back pain radiating down the posterior leg, worse with cold exposure, and reports frequent urination with a weak stream.

Walk through your full clinical reasoning: which channel(s) are implicated and why, and which points would you prioritize -- your answer must reference at least one Five Shu point AND the Back-Shu concept, not just name a channel.

MODEL ANSWER (check your reasoning against this after you've tried it yourself)

Posterior leg pain places this on the Bladder (Foot-Taiyang) pathway. Cold-worsened pain plus weak urination is a Kidney Yang deficiency pattern -- and BL/KI are the paired Water-element channels, so both are implicated even though only one shows up in the chief complaint. Point selection: BL40 Weizhong (He-Sea point AND a Command Point for the back -- "back points, look to Weizhong") for the local/pathway pain; BL23 Shenshu, the Back-Shu point of the Kidney, to address the underlying Yang deficiency driving the urinary symptom and cold sensitivity. This is the Back-Shu logic in action: you're not just needling where it hurts, you're using BL's Back-Shu series to reach Kidney systemically through a different channel's points.

CASE 2

SOLO level: Relational

A patient's symptoms don't fit one channel: shoulder pain radiating to the thumb, plus intermittent lower abdominal discomfort with bloating.

Using the circuit hand-off logic (not just "which channel runs there"), explain how LU, LI, ST, and SP could all be relevant to this ONE patient, and name the hand-off points connecting them.

MODEL ANSWER (check your reasoning against this after you've tried it yourself)

This is the full Anterior/Outer Circuit. Shoulder-to-thumb pain sits on LU (Hand-Taiyin, chest->hand) -- LU11 is at the thumb. LU hands off to LI at the fingers (LU7 branch meets LI1), and LI runs hand-> head, ending near the face (LI20). LI20 is also a crossing point where ST begins (ST's pathway truly starts at LI20, not ST1) -- ST then runs head->foot directly through the abdomen, which is exactly where the bloating shows up. ST hands off to SP at the toes (ST42/SP1), and SP runs foot->chest, completing the circuit back near where it started. So a single patient's shoulder pain AND abdominal bloating aren't necessarily two unrelated complaints -- they can be two points on the same continuous circuit.

CASE 3

SOLO level: Extended Abstract

A patient reports irregular menstruation, chronic low back pain, and describes a sensation of “Qi rushing upward” into the chest, worse with stress.

Which Extraordinary Vessel is most implicated, which confluent point pairing would you select, and explain WHY that pairing works mechanistically -- not just that it's the textbook combination.

MODEL ANSWER (check your reasoning against this after you've tried it yourself)

“Qi rushing upward” plus menstrual irregularity points to Chong Mai -- classically the Sea of Blood and Sea of the 12 Meridians, and specifically associated with rebellious Qi counterflow. Chong Mai's confluent point is SP4 Gongsun, paired with PC6 Neiguan (Yin Wei Mai). The pairing isn't arbitrary: SP4 is ALSO SP's own Luo-Connecting point, and PC6 is ALSO PC's own Luo-Connecting point -- so this combination doubles up on two mechanisms at once (Luo + Confluent) rather than working through just one. That's also why Dr. Zhang flags SP4+PC6 as the single most clinically-used confluent pairing: it treats chest, heart, AND stomach complaints together, which maps directly onto “rushing upward into the chest” as a presenting complaint.

CASE 4

SOLO level: Extended Abstract

No patient case this time -- a synthesis question instead.

Without looking anything up: name every point that is BOTH a Luo-Connecting point AND a confluent point opening an Extraordinary Vessel. Then explain why “double-duty” points like these are disproportionately high-yield for exam purposes.

MODEL ANSWER (check your reasoning against this after you've tried it yourself)

Four points do double duty: LU7 Lieque (Luo of LU, opens Ren Mai), SP4 Gongsun (Luo of SP, opens Chong Mai), PC6 Neiguan (Luo of PC, opens Yin Wei Mai), and SJ5 Waiguan (Luo of SJ, opens Yang Wei Mai). The other 4 confluent points -- SI3, BL62, KI6, GB41 -- are NOT simultaneously Luo points of their own channel. Why these matter disproportionately: a question about any ONE of these 4 points can be asked from at least two different angles (“what's LU's Luo point” vs “what opens Ren Mai”) and both have the same correct answer -- which means they're statistically more likely to appear on any given exam, in some form, than a point that only belongs to one category.

CASE 5

SOLO level: Relational

A patient has both heel pain and low back pain, and separately mentions feeling anxious and having trouble sleeping.

Is there a single-channel explanation that accounts for all three complaints, or does this genuinely require two different channels? Justify your answer using pathway logic, not just symptom-matching.

MODEL ANSWER (check your reasoning against this after you've tried it yourself)

Heel pain and low back pain alone could both be explained by Kidney (KI's pathway curves directly behind the medial malleolus and through the heel, and KI/BL are the paired Water channels covering low back). But anxiety and sleep disruption are not primarily a Kidney presentation in this system -- that's a Heart pattern (Shen disturbance). The honest answer is that this DOES require two channels: Kidney for the heel/low-back pathway complaints, and Heart for the sleep/anxiety complaint, with the Kidney-Heart (Water-Fire) relationship being the physiological reason both can appear together in the same patient rather than a coincidence. The exam trap this case is built to catch: forcing every symptom in a vignette into one channel just because two of the three complaints matched it.